

Business Transformation Charter

A practical operating model for connecting five clinics, improving patient continuity and giving leadership the visibility needed for sustainable growth.

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DOCUMENT	Business Transformation Charter Working draft for review
BUSINESS	Five-clinic private healthcare group · West Midlands

5	~£2m	~£3k
clinics	annual revenue	monthly no-show + rebooking leakage

Source basis

Prepared from the Brightpath case brief and the supplied charter template. The structure follows the template's charter logic, while the wording, layout and financial treatment have been redesigned for this report.

Executive summary

Brightpath Health Clinic has strong customer demand and a growing five-clinic footprint, but its operating model has not kept pace with that growth. The case brief identifies fragmented scheduling, disconnected patient records, inconsistent marketing, weak cross-clinic coordination, and limited management information as the main barriers to scalable growth. The founder's stated priority is to strengthen the operating engine before adding a sixth or seventh location.

This charter translates those case findings into a practical transformation agenda: create one operational view of appointments, establish a compliant central patient-record environment, automate routine patient communications, coordinate capacity across locations, standardise marketing activity, and give leadership timely performance information. The exact implementation budget and final delivery dates are not specified in the case brief and are therefore left for approval rather than invented.

Transformation objectives

ID	Objective	What changes	Success direction
01	Unify scheduling	Move all five clinics toward one scheduling process and system.	Reduce double-bookings, missed appointments and rebooking leakage.
02	Create one patient record	Establish a single GDPR-compliant source of truth for patient information.	Enable continuity when patients use more than one clinic or service.
03	Introduce management intelligence	Build a practical dashboard covering bookings, revenue and patient satisfaction.	Give the founder timely visibility instead of relying on delayed reports.
04	Automate routine communication	Introduce confirmations, reminders and follow-up workflows.	Reduce avoidable administrative effort and improve the patient journey.
05	Connect the five locations	Make spare capacity and referral opportunities visible across the network.	Retain more activity inside Brightpath instead of referring patients externally.
06	Strengthen marketing control	Use one coordinated content calendar and track which activity generates bookings.	Reduce dependence on fragmented local activity and one freelancer's availability.

Scope, assumptions & constraints

The charter protects the transformation from becoming an undefined growth project. The case brief is clear about the operational problems; the items below translate those problems into a controlled delivery boundary.

In scope

Included work
Central scheduling design and rollout across Birmingham, Wolverhampton, Coventry, Solihull and Walsall.
Migration/consolidation of active patient information into a GDPR-compliant central environment.
Daily or near-real-time management reporting for bookings, revenue and patient satisfaction.
Automated appointment confirmations, reminders and follow-up messages.
Cross-clinic capacity visibility and a defined internal referral process.
A shared marketing calendar, consistent brand assets and channel-level booking attribution.
Staff training, workflow documentation and adoption support.

Out of scope

Excluded / deferred
Opening the sixth or seventh clinic as part of this transformation.
Clinical treatment or care-protocol changes.
Replacing the transformation with a purely marketing-led growth programme.
Assuming a final software vendor, implementation cost or contract before procurement is completed.

Key assumptions

Planning assumption
The five existing clinics remain the transformation boundary unless the sponsor approves a change.
Clinical teams participate in workflow design and adoption activities.
Patient information is handled in line with applicable UK healthcare data-protection requirements.
The founder provides timely decisions on scope, priorities and investment.
Any final platform choice must support multi-location scheduling, central records and reporting.

Stakeholder map

Brightpath's transformation is primarily an operating-change programme, not a software installation. The people who use the current routines every day therefore need a stronger voice than their formal job titles might suggest.

Stakeholder	Interest	Influence	Engagement
Dr. Amara Okonkwo — Founder	High	High	Weekly decision forum; approve priorities and major changes.
Office manager	High	High	Operational co-lead; coordinate site readiness and adoption.
Clinicians across five clinics	High	High	Design workflows, pilot new processes and champion adoption.
Reception / administrative staff	High	Medium	Hands-on users; involve them in testing and training.
Marketing freelancer	Medium	Medium	Align content calendar, brand assets and booking attribution.
Patients	High	Low	Communicate changes clearly; protect continuity and ease of booking.
External technology / implementation partner	Medium	Medium	Contracted delivery against agreed requirements and milestones.

Engagement principle

Make the new process easier than the old one. Clinicians and reception teams should help shape workflows, test the pilot and identify friction before the model is expanded across the network.

Transformation roadmap

A phased delivery model is recommended so Brightpath can prove the operating model before committing the whole network. The sequence below is a planning recommendation, not a set of dates stated in the case brief.

Phase	Focus	Key outputs
1. Diagnose & design	Confirm requirements and baseline performance.	Process maps, requirements, data inventory, KPI definitions, governance.
2. Configure & prepare	Select/configure the operating platform and prepare data.	Configured workflows, cleaned data, migration plan, training materials.
3. Pilot & refine	Test the new model in a controlled clinic environment.	Pilot feedback, corrected workflows, adoption measures, go/no-go decision.
4. Network rollout	Extend the operating model across all five locations.	Central scheduling, records, automation and capacity visibility in use.
5. Stabilise & measure	Embed the new way of working and track outcomes.	Dashboard, SOPs, benefits review, lessons learned and expansion-readiness assessment.

Delivery method

Use a gated, iterative approach: requirements and data controls should have clear approval points, while training and workflow adoption should be refined through short feedback cycles. This keeps governance disciplined without forcing every clinic to change at exactly the same pace.

Critical dependencies

Dependency
Clear ownership of patient-data quality and privacy controls.
A platform that supports all five locations rather than another local workaround.
Protected time for the founder, office manager, clinicians and reception teams.
A pilot that is representative enough to expose real workflow problems before wider rollout.

Success criteria

Success should be demonstrated through observable operational behaviour and management evidence, not simply by proving that software was installed.

Outcome	Evidence of success	Owner
Scheduling	All five locations operate from a coordinated scheduling process; double-booking and no-show/rebooking leakage trend downward from the case baseline.	Office manager
Patient records	Active records are centrally accessible and new records are no longer being created in disconnected local stores.	Project lead / clinical representatives
Management visibility	Leadership can view bookings, revenue and patient satisfaction without waiting weeks for a retrospective report.	Founder / office manager
Automation	Confirmation, reminder and follow-up workflows run consistently for agreed appointment types.	Operations lead
Cross-clinic capacity	Staff can see available capacity and use an agreed internal referral route.	Clinic leads
Marketing	One calendar is used across the network and bookings can be associated with marketing channels.	Marketing lead
Expansion readiness	A documented evidence base supports the decision on whether a sixth and seventh location are operationally viable.	Founder

Minimum definition of done

The transformation is considered operationally embedded when the five clinics are working from the agreed processes, leadership has current management information, routine patient communications are automated where appropriate, and the organisation can use evidence—not guesswork—to assess expansion readiness.

Risk register

The largest threats are organisational as well as technical. Adoption, data quality, decision speed and ownership can undermine a good platform if they are not managed deliberately.

Risk	Likelihood	Impact	Response
Clinician resistance to changing established routines	High	High	Use site champions, involve clinicians in design, pilot before network-wide rollout, and provide practical training.
Poor-quality or duplicated patient data	Medium	High	Audit and cleanse data before migration; define ownership and validation checks.
Founder bandwidth limits decisions	Medium	High	Use a standing decision forum and escalate only decisions requiring sponsor input.
Operational dependency on one office manager	Medium	High	Document processes, nominate backup ownership and distribute project knowledge.
Technology implementation does not fit clinic workflows	Medium	Medium	Use requirements-led selection, demonstrations, pilot testing and formal acceptance criteria.
Data-protection or privacy gap	Low	High	Complete appropriate privacy assessment and controls before patient data is migrated.
Marketing remains fragmented	Medium	Medium	Set one calendar, one asset library and clear ownership for publishing and attribution.

Risk ownership rule

Every material risk should have one named owner, one mitigation action and a review date. Risks that change the approved scope, cost or target outcome should move into formal change control.

Business value & investment logic

The case brief does not provide an approved transformation budget, vendor pricing, implementation fee, or formal ROI/payback calculation. This report therefore does not copy the template's financial figures. Instead, the ~£3,000 monthly no-show/rebooking leakage is retained as a source baseline, with an annualised indicator of approximately £36,000. That figure is a diagnostic extrapolation, not a promised benefit.

Known case indicator	What it tells us	Treatment in this charter
~£3,000/month lost to no-shows + rebookings	Material recurring operational leakage.	Use as the baseline for benefits tracking; annualised indicator is ~£36,000 if the monthly rate persisted.
~£2m annual revenue	Brightpath has meaningful scale and needs stronger management information.	Prioritise decision-grade reporting rather than more spreadsheets.
Five clinics / two more considered	Growth is constrained by operational fragmentation.	Make expansion readiness an explicit outcome, not an assumption.

Value themes to track

Value area	Measure
Revenue protection	No-show and rebooking leakage versus the ~£3,000/month case baseline.
Capacity utilisation	Internal referrals and use of available clinician capacity across sites.
Administrative efficiency	Time saved through central records, automation and fewer duplicate processes.
Management quality	Speed and reliability of access to bookings, revenue and patient-satisfaction information.
Growth readiness	Whether the five-clinic model can support additional locations without recreating current fragmentation.

Governance, change control & approval

The project should remain governed as a business transformation with clear decision rights. Scope changes, major technology decisions and changes to the approved operating model should be recorded rather than agreed informally.

Mechanism	Cadence	Purpose
Sponsor decision forum	Weekly during active delivery	Resolve scope, priority and investment decisions quickly.
Delivery review	Every two weeks	Review progress, adoption issues, risks and next actions.
Management dashboard	Daily / agreed refresh	Provide current operational and commercial visibility.
Risk review	Monthly	Reassess risk exposure and mitigation ownership.
Change control	Before material scope changes	Record impact on cost, timing, people and outcomes before approval.
Phase gate	At major transition points	Confirm readiness before moving from design to pilot and rollout.

Approval

Approval confirms that the charter is the working basis for the transformation and that material changes will be documented and reviewed.

Role	Name	Signature / date
Project Sponsor	Dr. Amara Okonkwo	_____
Project / Transformation Lead	Michael Utomi	_____
Operations Lead	Brightpath Office Manager	_____

Prepared by Michael Utomi · Brightpath Business Transformation Charter · Version 1.0